

A Shared Vision for the Future

Rare and Complex Epilepsies in Australia

PREPARED FOLLOWING THE DEE ROUNDTABLE
AUGUST 2026

COLLABORATION
EVIDENCE
IMPACT



Executive Summary

Developmental and Epileptic Encephalopathies (DEEs) are severe, genetically driven, neurodevelopmental disorders. Epilepsy is only one feature of this complex, multi-system and lifelong condition. Treating DEEs as a subset of epilepsy understates their impact and produces the wrong system response. They require coordinated action across health, disability, education, research and social systems.

Throughout this Report, DEEs are also referred to as Rare and Complex Epilepsies, reflecting the severity and breadth of these conditions rather than narrowing them to a label. The rationale for this shift is set out in the Introduction.

Australia does not lack expertise. It lacks the national mechanisms to align and deploy it. Strengths in clinical care, diagnostics, research, disability, education, policy and advocacy are disconnected. Families pay the price for that fragmentation, through delayed diagnosis, inconsistent multidisciplinary care, disability and education systems that cannot respond to complexity, and inequitable access to emerging precision therapies. These are not isolated service gaps. They are the predictable consequences of a system without national leadership.

The current Senate Inquiry and heightened political attention create a time limited window for reform. That window must be used to secure national recognition, investment and accountability for Rare and Complex Epilepsies. These rare disease communities have shown that a national voice can convert fragmented effort into policy change.

The DEE Roundtable 2020 brought together patient advocates, clinicians, researchers, policymakers, industry and other sector leaders to determine what must happen next. A mandate emerged: Australia must establish a national organisation with the authority, representation and capability to lead the sector.

The National Rare and Complex Epilepsies Collaborative is the sector's chosen mechanism for national coordination. It will bring stakeholders together to align Australia's fragmented ecosystem, agree shared priorities, inform policy and system reform, accelerate research translation, improve care and strengthen advocacy for equitable access to emerging therapies. Its task is not simply to convene. It is to enable expertise and consensus to inform coordinated national action across health, research, disability and education.

Executive Summary

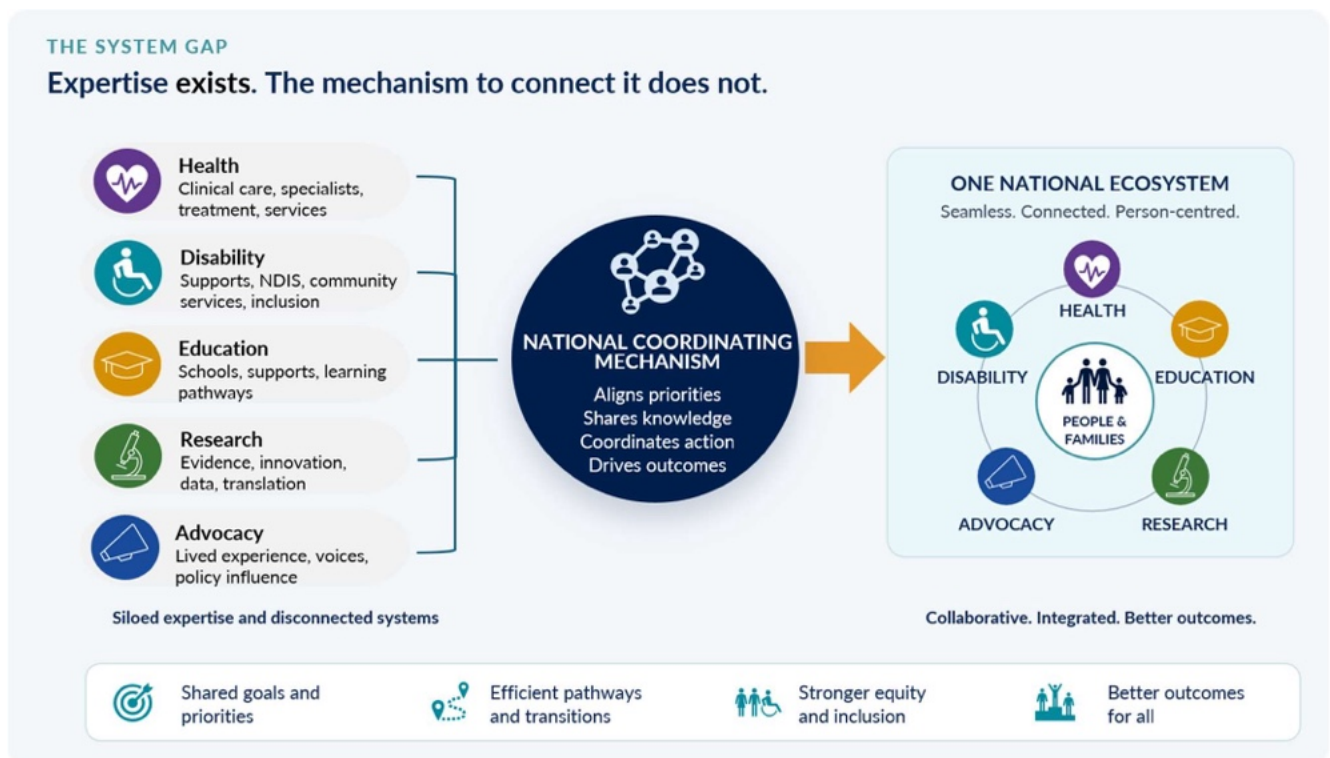


Figure 1. Australia's Rare and Complex Epilepsies coordinated through the Collaborative.

The legitimacy of the Roundtable process bringing together stakeholders from across the Complex Epilepsies landscape to collaboratively design the sector's future is central to the strength of the Roundtable consensus and a critical enabler of the reforms it calls for.

This Report is not a record of discussion. It captures the sector consensus and calls for a move from fragmented goodwill to coordinated national delivery.

Purpose

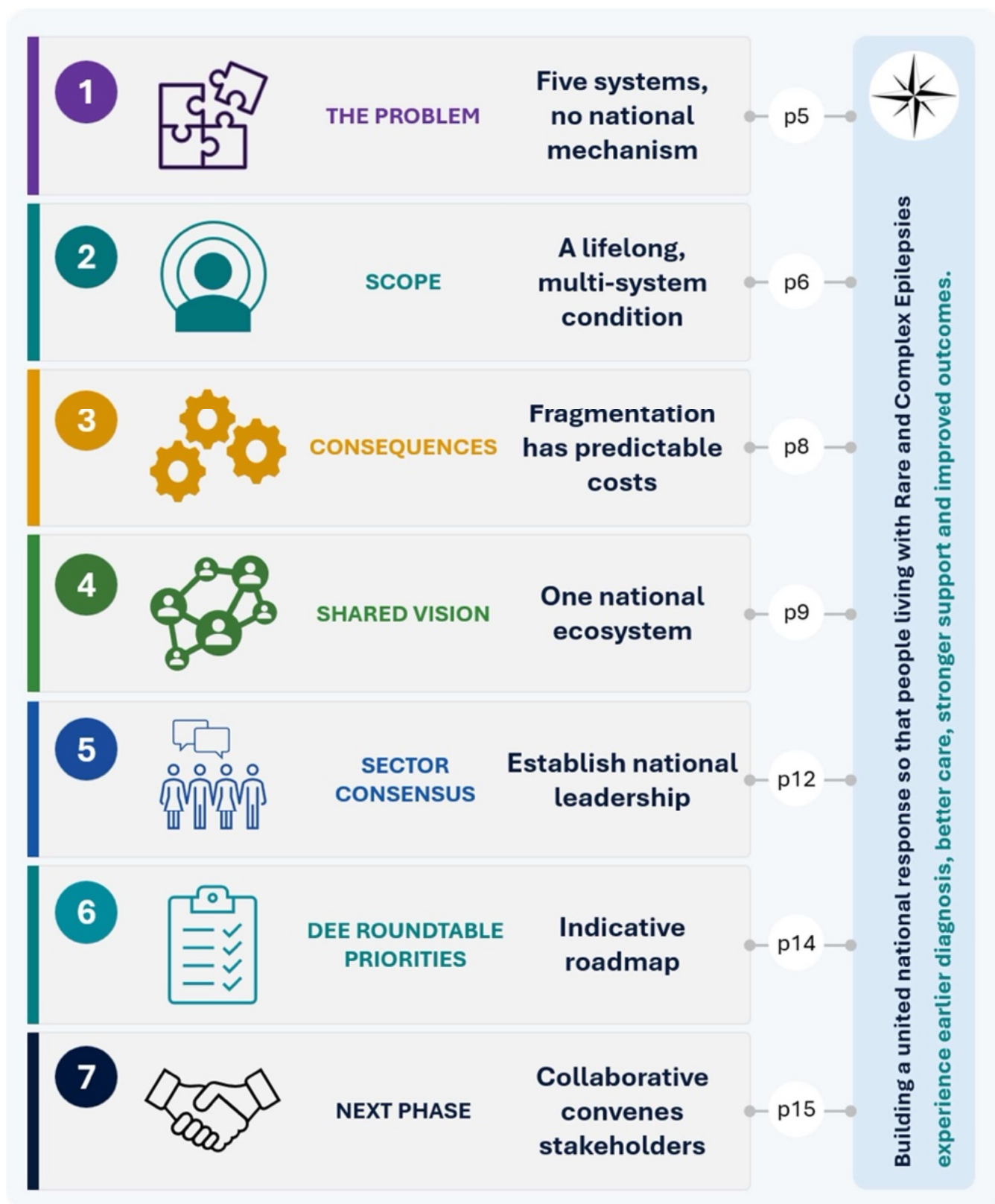
This Report establishes the basis for coordinated national action, recording the sector consensus reached by stakeholders in the Rare and Complex Epilepsies sector during the Roundtable, providing a direction for the Steering Committee and the broader sector to develop the mechanism for national coordination and provides the evidence base underpinning the Roundtable's recommendations, the foundation for advocacy and funding proposals to further develop the collaboration model.

The purpose is practical: to turn consensus into decisions, decisions into funded work, and funded work into measurable improvements for people and families.

A companion Methodology Report sets out the full design, facilitation approach and data collection methods behind this Report's findings.

Report at a glance

From system gaps to a connected response





The Problem



Across health disability education and social systems Rare and Complex Epilepsies folded into the broader category of epilepsy either the evidence nor the lived experience of families supports this Families are not falling through gaps in one system they are falling through gaps in every system and none of these systems can meet their needs in isolation coordinated national leadership can address the cross system fragmentation currently experienced by people living with Rare and Complex Epilepsies Pierce et al 2019

The insights in this report drawn from the Roundtable and the consultation process tell us why it set out why

National Coordination

No single system can meet the needs of people living with Rare and Complex Epilepsies in isolation The sector is fragmented with duplication of effort across organisations and groups missed opportunities for shared leadership and no mechanism to convene or coordinate stakeholders Progress is fragile and uneven dependent on individual champions rather than system design

National Voice

The Rare and Complex Epilepsies landscape currently lacks a recognised authoritative voice to represent all stakeholders and particularly families Advocacy is fragmented government engagement is inconsistent and policy settings do not reflect the complexity of these conditions Establishing a national coordinated organisation is essential to influence policy secure funding and drive systemic reform



Figure 2. Five systems impacting patients and families



Scope of the Challenge



DEEs are the most severe group of epilepsies. The International League Against Epilepsy formally recognised DEEs as a distinct category in its 10th classification, recognising developmental consequences arising directly from the effect of the genetic mutation to the effect of the frequent epileptic activity on development. Scheffer et al.

Individuals with DEE experience a wide range of multi-morbidities in addition to early-onset, usually drug-resistant epilepsy. These include intellectual disability, autism, psychiatric and behavioural consequences, movement and musculoskeletal disorders, feeding and gastrointestinal problems, sleep disturbances, and an elevated risk of sudden unexpected death in epilepsy (SUDEP). DEEs are lifelong conditions consistently associated with developmental slowing or regression. Scheffer et al. Most are genetically driven, each individually rare, but collectively common. Management depends on the underlying cause.

Population-based data estimate that DEEs collectively affect approximately 7 in 10,000 Australian children. (Poke et al., 2016)

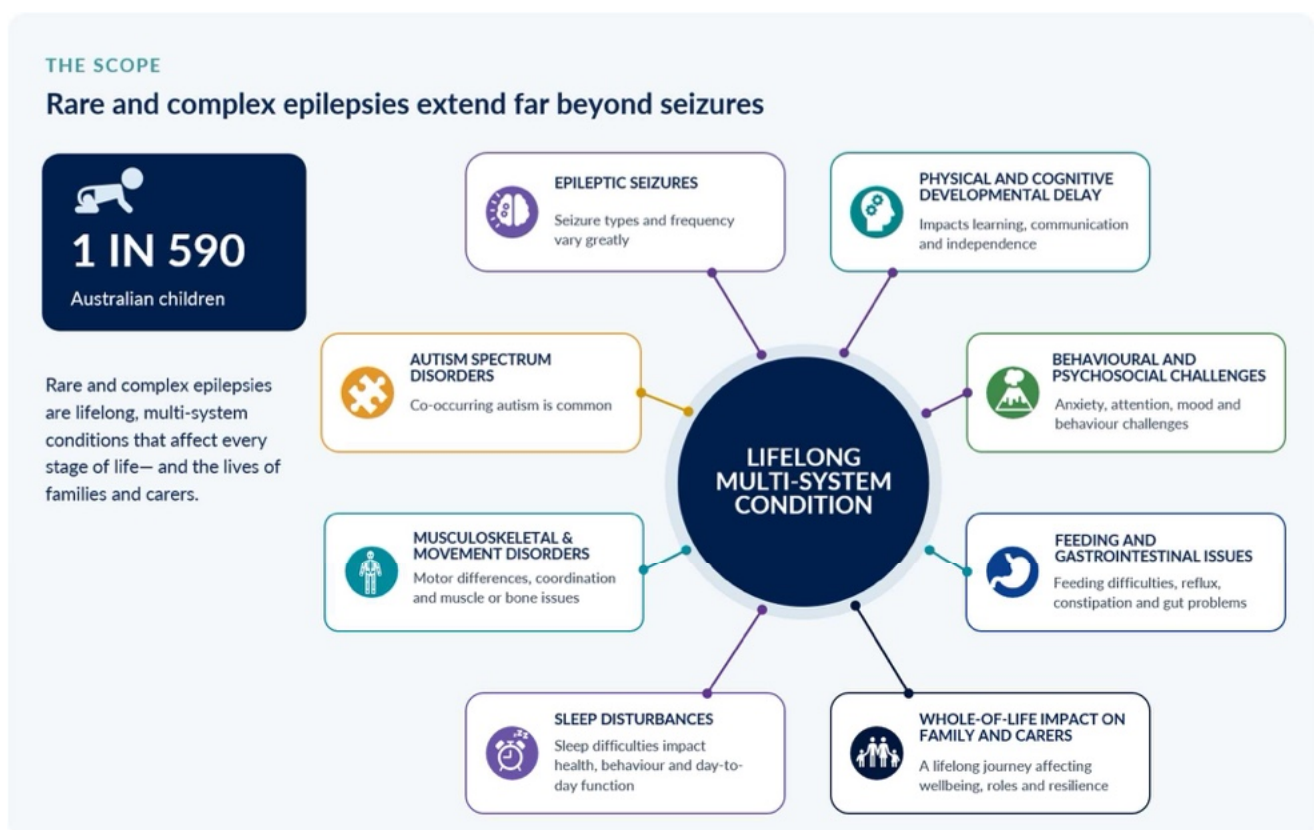


Figure 3. Persons born with a rare and complex epilepsy have a lifelong impact on them and their families.



Scope of the Challenge



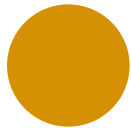
A life long multi system condition

This classification matters because it makes clear that DEEs impose a lifelong burden on affected individuals, families and society and because it defines what good care looks like. Managing a DEE well is not the same task as managing epilepsy well. The complexity and system nature of DEEs requires lifelong, holistic and coordinated multidisciplinary management that extends beyond seizure control. (Scheffer et al)

Throughout this Report, Developmental and Epileptic Encephalopathies (DEEs) remain the clinical classification. However, the Report also adopts the broader term Rare and Complex Epilepsies in recognition of a key area of consensus from the DEE Roundtable that while clinically important, it is not well understood outside specialist settings and does not convey the severity, complexity or whole of life impact of these conditions.

The use of Rare and Complex Epilepsies is therefore a deliberate public and policy framing. It is intended to improve recognition among governments, funders, service systems and the broader community without altering or replacing the clinical classification of DEE.

This distinction matters because the way these conditions are framed influences how they are recognised and responded to. When they are understood primarily through the lens of seizures, the broader developmental, cognitive, behavioural, communication, movement and disability impacts can be overlooked, contributing to the fragmentation described throughout this Report.



Consequences of Fragmentation



Fragmentation has predictable costs

Recognition beyond Epilepsy

Rare and Complex Epilepsies cannot be effectively addressed within an epilepsy only framework. The current framing narrows care to seizure control and obscures the developmental, behavioural, cognitive, psychosocial and disability needs that define these conditions. This misalignment shapes every part of the system: clinical pathways focus on seizure rates rather than whole condition management; policy settings fail to reflect multi-system impact and are built out of paediatric-centric structures entirely. Establishing Rare and Complex Epilepsies as a distinct category is foundational to any coordinated national response.

I feel like I'm constantly having to explain that it's not just epilepsy. It's intellectual disability. It's language delay. It's behavioural challenges.

parent DEE Roundtable family consultation

Coordinated lifelong Support

Families described overwhelming complexity and a lack of coordinated support across every stage of the journey: trauma from first seizures, repeated crises, years of being disbelieved, exhaustion, burnout, post-traumatic stress disorder, PTSD, financial strain, the toll of retelling their story to professionals who do not communicate with each other. The consequences are profound, inequity driven by geography, socioeconomic status, cultural background and health literacy, delayed access to supports and avoidable distress.

The biggest challenge on a day-to-day basis is that our child needs full supervision all day, every day.

parent DEE Roundtable family consultation

Diagnosis Treatment and Research

Access to diagnosis and treatment varies significantly across Australia. Families often rely on their own advocacy to drive investigations, secure genomic testing or obtain specialist appointments. Diagnostic pathways differ between states; adults and late-onset presentations are particularly disadvantaged, and comorbidities such as sleep, gut, mental health and cognition are frequently overlooked. Limited access to multidisciplinary teams and centres of excellence slows trial readiness and restricts participation in emerging therapies.

Integrated Systems

DIS pathways are slow, unpredictable and not tailored to the complexity of Rare and Complex Epilepsies. Schools lack condition-specific knowledge, and community providers often misunderstand the condition entirely. Families from Culturally and Linguistically Diverse backgrounds, First Nations communities, and rural and remote areas face additional barriers. Fragmented and siloed data systems prevent coordinated care.

Research Translation

Australia has world-class research capability, particularly in genomics, natural history studies, and early phase drug development, but research efforts are fragmented, with duplication of effort, inconsistent coding and classification, and limited visibility of who is doing what. Regulatory frameworks are not suited to rare and ultra-rare diseases, and biobank and infrastructure is inadequate.



Shared vision



The Roundtable gave key stakeholders from across the Rare and Complex Epilepsies landscape the opportunity to unite around a shared vision for the future. The strength of consensus among attendees as to what that future looks like underpins the shared direction set out in this vision.

Central to this vision is the *National Rare and Complex Epilepsies Collaborative* – a national organisation that addresses the System Gap described above, providing leadership and stakeholder representation for the sector.

The Collaborative will bring together patient groups, clinicians, researchers, governments and advocacy organisations within a nationally coordinated, equitable and person-centred Rare and Complex Epilepsies ecosystem. Through an integrated ecosystem model that shares knowledge and expertise, shares leadership and mobilises champions across clinical care, research, policy and advocacy, it will enable stakeholders to align fragmented efforts and support progress toward consistently high quality care, world class research and development, education and awareness, and lasting system reform. The proposed Collaborative will seek government support to advance equity for people and families affected by Rare and Complex Epilepsies, including recognition, investment and access to support on par with other lifelong conditions.

THE SHARED VISION

One national collaborative connecting people, evidence and action



Working together to ensure people living with Rare and Complex Epilepsies and their families have the best possible outcomes—
everywhere in Australia.

Figure 4. The Australian Rare and Complex Epilepsies Collaborative connecting people, evidence and action.



Shared vision



People and Families People living with Rare and Complex Epilepsies and their families experience a system that is coordinated, navigable and centred on their needs across the lifespan. They have access to specialist nursing, care coordination and navigation that supports seamless pathways across diagnosis, treatment, disability, education and social services.

People and families can draw on a single, trusted source of information about Rare and Complex Epilepsies, from understanding conditions and care pathways to accessing emerging and precision therapies. Digital tools improve access and provide practical resources, including a Rare and Complex Epilepsies toolkit designed with and for people and families.



Clinicians Clinicians work within connected specialist networks and a nationally consistent, co-designed model of care supported by multidisciplinary teams and aligned with international standards. Timely genomic diagnostics are available nationally, enabling earlier and more accurate diagnosis of Rare and Complex Epilepsies and supporting appropriate care, research participation and access to emerging therapies.

Clinicians have access to best practice guidance and clear pathways across health, disability and community services. Geography, institutional setting and local resourcing no longer determine whether people and families can access coordinated, high quality care.



Researchers Researchers work within a connected, data-enabled national research ecosystem with shared priorities and infrastructure. Shared datasets, meaningful measures of real world impact and quality of life, and improved disease classification and coding enable deeper understanding of Rare and Complex Epilepsies and more targeted research.

Researchers can collaborate across institutions, disciplines and jurisdictions with stronger pathways from discovery to clinical translation. Australia is internationally recognised for aligned research priorities, trial readiness, genomics capability and meaningful involvement of people and families in research.



Shared vision



Policymakers Policymakers have a clear national understanding of Rare and Complex Epilepsies supported by consistent definitions evidence and lived experience that inform policy funding service design and health disability pathways

They have established mechanisms for engaging with the Collaborative and accessing coordinated sector expertise Stronger connections across Commonwealth state and territory governments the DIS and other relevant systems support more coherent policy and reduce fragmentation Champions within government help translate evidence and identified priorities into sustained reform



Advocates Advocates work across organisations communities and jurisdictions around shared priorities for Rare and Complex Epilepsies Alignment on core messages and policy priorities strengthens their ability to engage decision makers while preserving the distinct voices expertise and identities of individual organisations and communities

Advocates have clear mechanisms for bringing lived experience and community priorities into the Collaborative and for taking shared evidence and messages back to their own communities Together they strengthen the national voice for Rare and Complex Epilepsies and increase its influence across local state and Commonwealth policy



Industry Partners Industry partners see Australia as an attractive environment for responsible research clinical development and investment in Rare and Complex Epilepsies Connected clinical and research network appropriate access to research cohorts national data capability and trial ready infrastructure provide a strong environment for developing and evaluating new therapies

Industry partners engage transparently and responsibly with researchers clinicians people and families with clear principles governing collaboration and conflicts of interest Strong partnerships support investment in Australian research and clinical trials and increase opportunities for Australians to participate in the development of emerging therapies



Sector Consensus



THE SECTOR CONSENSUS

The minimum platform for reform

01

ESTABLISH NATIONAL LEADERSHIP

A permanent mechanism across organisations and jurisdictions

02

CONTRIBUTE TO NATIONAL ACTION PLANNING

A broader epilepsy plan with a dedicated rare and complex plan

03

AGREE PRIORITIES THROUGH THE COLLABORATIVE

Relevant stakeholders determine how priorities are taken forward

The Roundtable reached alignment across lived experience, clinical care, research, policy, advocacy, and industry.

The recommendations below are not a menu of options. Together, they form the minimum platform for reform.

THE INTEGRATED ECOSYSTEM MODEL

A national hub that aggregates expertise and coordinates action

A Rare and Complex Epilepsies coordinating organisation will play a system integrator role. It will work with a network of partner organisations to collaboratively set national priorities, provide a single voice to all stakeholders, coordinate action and align funding to projects addressing the greatest needs.



Figure 5. The integrated ecosystem model: The R organisation convenes the Collaborative and wor priorities and then provides a consistent voice across the sector.



Sector Consensus



National Collaboration Model

Australia requires a permanent mechanism that can provide leadership across organisational jurisdictional boundaries. The Collaborative will bring together the existing strengths of support organisations to agree shared priorities, contribute to consistent standards and hold the mechanism together when individual projects, leaders or funding cycles change.

Contributing to National Epilepsy Action Planning

The Collaborative will contribute to the development of a broader national Epilepsy Action Plan, including a dedicated Rare and Complex Epilepsies Action Plan sitting within it. Through the Collaborative, stakeholders would determine how the priorities identified by the DEE Roundtable should inform planning across health, disability, education, research and community systems. This could include a shared understanding of national standards of care, diagnostic pathways and models of support, equitable access to genomic testing, multidisciplinary teams and centres of excellence, inform priorities for investment, workforce development and data infrastructure. It would address the postcode lottery currently experienced by people living with Rare and Complex Epilepsies, embed lived experience insights into system design and evaluation.

National Organisation

From the collective voices of the Rare and Complex Epilepsies community, a strong sector agreement has emerged to establish a national organisation that provides leadership and stakeholder representation for the sector. A Rare and Complex Epilepsies coordinating organisation will convene the Collaborative with partner organisations and would serve as the unified voice for Rare and Complex Epilepsies, providing authoritative advice to governments, coordinating stakeholder engagement across systems and supporting the sector to contribute to the broader national Epilepsy Action Plan and the dedicated Rare and Complex Epilepsies Action Plan within it.

The Collaborative would not duplicate existing strengths; it would build on them, operating as an integrated ecosystem model described above. It would ensure that undiagnosed families, adults and children from diverse backgrounds, including CALD, First Nations and rural and remote communities, are included in national coordination.

Roles, Responsibilities and Governance

The Rare and Complex Epilepsies coordinating organisation should be established as an independent, non-government entity with multi-stakeholder governance. Its roles and responsibilities should include:

- R** Leadership and coordination across the Rare and Complex Epilepsies landscape
- R** Policy and systems engagement, including structured government liaison
- R** Development of national standards of care, diagnostic pathways and models of practice
- R** Coordination of research and data, including national registries and harmonised coding
- R** Workforce development, including training for clinicians, allied health, educators and providers
- R** Navigation and family support, including nurse navigators and peer system navigators
- R** Awareness and education, including national campaigns and co-designed toolkits
- R** Alignment with national and international frameworks

Governance should include lived experience, clinical experts, researchers, advocacy organisations, policy leaders, supported by advisory groups focused on specific domains such as research, care, disability, education and data.



The Roundtable identified priorities for a connected national response. The timeframes are indicative. Sequencing, implementation and accountability will be determined collaboratively with relevant stakeholders.

An indicative roadmap to national impact





Next Phase



From Consensus to Action

The consultation phase has established the basis for action. The next phase is to establish the Collaborative and bring stakeholders together to determine how the Roundtable priorities are taken forward.

The Steering Committee will support the transition to an interim establishment group with representation authority and capabilities required to establish the Collaborative. Immediate priorities are governance, stakeholder engagement, funding and government engagement.

The Committee will use the Senate Inquiry and the lead up to the 1st Federal commitments to national recognition, establishment, funding and government partnership. It will formalise sector participation through the integrated ecosystem model.

Funding is an immediate strategic priority. The Committee will pursue start-up investment to establish the Collaborative and enable stakeholder engagement on the Roundtable priorities, as well as a pathway to sustainable operating funding.

We will continue to communicate with the Rare and Complex Epilepsies sector and encourage you to reach out to the Steering Committee via the contact details below.

On behalf of the Roundtable Steering Committee

Email: edil@healthadvancerhomerct04021551014

Methodology

Background to the Roundtable

The DEE Roundtable 2020 built on the inaugural Roundtable held in 2013 at a national conference on Complex Epileptic Encephalopathies and its resultant publication *Guiding the way: Fulfilling the needs of children with developmental and epileptic encephalopathies* (Elada et al 2015). While the 2013 Roundtable brought together leaders in the Australian landscape for the first time, the momentum felt on the day was sustained by the Steering Committee members and other key opinion leaders as the strategic context had evolved enough for a further Roundtable to make a lasting difference.

The DEE Roundtable 2020 *From Insights to Impact* brought together patient advocates, clinicians, researchers, policymakers, industry and others to co-design practical collaborative actions to improve outcomes for families affected by Rare and Complex Epilepsies in Australia. Its objectives were:

- R** To identify and articulate the key challenges and priorities of families across the child and life course
- R** To define a set of clear, realistic and meaningful goals for advocacy across key areas
- R** To agree on a practical and coordinated pathway to deliver on these goals

This Report draws on a structured, multi-phase engagement process, analysis of the Complex Epilepsies landscape, consultation with parents and carers via questionnaire and focus group, and a full day Roundtable held in Brisbane on 10 May 2020 attended by 100 individuals from 20 organisations, guided by independent facilitator, Marie Littlefield.

Attendees worked through a series of facilitated workshops examining the current situation, the desired future state, and the barriers and drivers between them, before agreeing shared goals and an agreement to establish a national coordinating body.

The full design, facilitation approach, stakeholder selection process and data collection are set out in the companion DEE Roundtable 2020 Methodology Report.

Appendix

Acknowledgements

Acknowledgement of Country

The Steering Committee acknowledges Aboriginal and Torres Strait Islander peoples as the first Australians and recognises their cultures, histories and diversity and their deep connection to the land, waters and seas.

Roundtable Steering Committee

This project was led by a dedicated Steering Committee responsible for guiding the project's strategic direction, ensuring integrity in decision making and maintaining accountability throughout.



The DEE Roundtable 2026 Steering Committee

- Ris Pierce** Project Chair and Co-lead Director SC Australia and Strategic Rare Diseases Society
- Associate Professor Catherine Owen** Neurologist and Neurology Research lead Royal Children's Hospital Clinician Scientist Fellow Neuroscience Group Leader and Epilepsy Team Leader Murdoch Children's Research Institute
- athryn Milne** Project lead Rare Insight Advisory
- Associate Professor Elisabeth Palmer** Sydney Clinical Geneticist at Sydney Children's Hospitals Network
- Dr Neil Deacon** Founder Health Advance Connect
- Meagan Cross** Co-Founder and CEO FAST Australia Foundation for Angelman Syndrome Therapeutics
- Leah Duffy** Genetic Epilepsy Team Australia

Parents and Carers of People Living with DEEs

The Steering Committee sincerely thanks the parents and carers of people living with DEEs who generously shared their time and lived experiences at each stage of the project.

Sponsors

The Roundtable Steering Committee also thanks Lundbeck Australia Pty Ltd, SC Australia and C Australia Pty Ltd for their financial support towards the project as a whole, the Roundtable and the preparation of this Report.

The findings, sector consensus and recommendations in this Report reflect the consensus reached by Roundtable attendees and the broader consultation process. Sponsors provided financial support for the Roundtable and the preparation of this Report but had no role in determining its content, findings or recommendations.

Appendix Organisations Represented at the DEE Roundtable 0

Thirty four organisations from across the country were represented at the Roundtable. In addition, additional organisations and families with lived experience were involved in the consultation process but did not attend the Roundtable.

(Alphabetical order — 34 of 34 shown)

Alexion Pharmaceuticals	Queensland Children's Hospital
Austin Health	Queensland University of Technology
Autism Queensland Limited	Rare Diseases Society
Carers Queensland	Rare Voices Australia
Epilepsy Action Australia	SCA Australia
Epilepsy Queensland	Syngap Research Fund Australia
Epilepsy Tasmania	Sydney Children's Hospitals Network
Foundation for Angelman Syndrome Therapeutics	The Children's Hospital at Westmead
Genetic Epilepsy Team Australia	The Florey Institute of Neuroscience and
Genomics Australia	ETMental Health
Health Advance Connect	The Kids Research Institute Australia
In the A	The Pilepsy Foundation
Janus Pharmaceuticals	The Royal Children's Hospital
Junbeck Australia Pty Ltd	The University of Melbourne
Murdoch Children's Research Institute	The University of Queensland
Nabucodonosor	TC Australia Pty Ltd
Patient Voice Initiative	TS Sydney
	Victorian Clinical Genetics Service